jaundice :

Hyperbilibirunimia is most common and in most cases benign problem in neonates.

* indirect (unconjugated) → Potentialy neorotoxic
  (unbound to albumin ; free)
* direct (conjugated) → Signifies a Serous illness
  (bound to albumin)

jaundice observed approximately → %60 of term infants
→ %80 of Pre-term infants

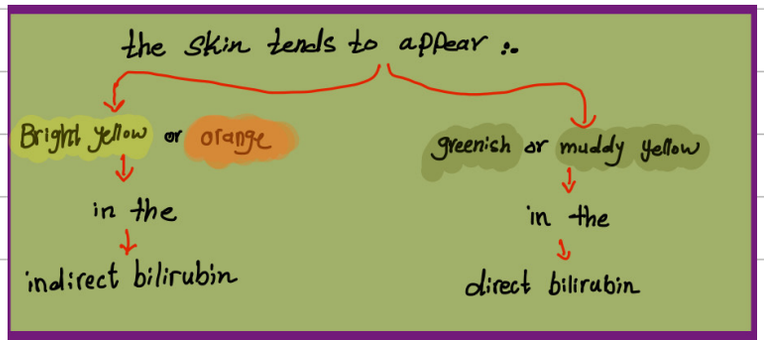
Etiology :

Fetal, placenta is the principle route of elimination of lipid Soluble bilirubin
adult stage, water-Soluble conjugated form excreted from :
((Hepatic cell → bile → billary System → GIT tract))

Unconjugated hyperbilirubinemia :

1. increase the load of bilirubin to be metabolized by liver.
2. reduce the activity of transferase enzyme
3. block transferase enzyme
4. absence or decresed ammount of enzyme or to reduction of bilirubin uptake by liver.

the toxic effects from elevated levels of unconjugated bilirubin are incresed by
- factors that reduce retention (excrition) of bilirubin in the Circulation
- factors that increse permabillity of the blood brain barrier.



- jaundice that first appears in the 2nd or 3rd day are usually physiologic

- jaundice appearing after 3rd day or within 1st week should suggest Becterial sepsis or urinary tract infection

Risk factors for indirect Hyperbilubinema :

1. maternal diabetes
2. prematurity
3. Drugs
4. Polycythemia
5. male sex
6. 21-trisomy (down Syndrome)
7. oxytocin induction
8. breast feeding, weight loss
9. Sibling who has physiologic jaundice
10. delayed bowel movement